

FINAL CONSOLIDATED PROTECTED DISCLOSURE

TO THE GEMS BOARD OF TRUSTEES

DATE: 13 April 2026

TO:

- The Principal Officer, Government Employees Medical Scheme (GEMS)
- Chairperson, GEMS Board of Trustees
- Chairperson, GEMS Risk & Audit Committee
- Chairperson, GEMS Clinical Governance Committee

COPY FOR RECORD (NOT FOR RESPONSE):

- Parliamentary Portfolio Committee on Health
- Public Protector of South Africa
- Council for Medical Schemes (CMS)
- South African Nursing Council (SANC)

BY EMAIL,

SUBJECT: FINAL CONSOLIDATED PROTECTED DISCLOSURE — SYSTEMIC CLINICAL RISK, GOVERNANCE FAILURE, RETALIATION, UNLAWFUL FINANCIAL DEDUCTION, POST-EMPLOYMENT MISREPRESENTATION, AND ONGOING OCCUPATIONAL DETRIMENT

PREAMBLE

I, Wilbur William Matthee, a Registered Nurse Specialist (SANC No. 15831886 — currently deregistered as a direct consequence of the Respondents' conduct), Postgraduate Diploma in Health Services Management (NQF Level 8), and forensic governance practitioner (Founder, EthicHawks), submit this Final Consolidated Protected Disclosure to the GEMS Board of Trustees.

This disclosure is made under the Protected Disclosures Act 26 of 2000 (PDA), read with:

- The Medical Schemes Act 131 of 1998 (GEMS's fiduciary and statutory responsibility for its administrator, Medscheme)
- The Nursing Act 33 of 2005 (my professional obligations as a Registered Nurse)
- The National Qualifications Framework Act 67 of 2008 (my statutory obligation to report clinical risk as an NQF Level 8 graduate)
- The Labour Relations Act 66 of 1995 (protection against retaliation for protected disclosures)
- The Employment Equity Act 55 of 1998 (protection against discrimination on grounds of disability, sexual orientation, and gender identity)
- The Basic Conditions of Employment Act 75 of 1997 (unlawful salary deduction)
- The Unemployment Insurance Act 63 of 2001 and Unemployment Insurance Contributions Act 4 of 2002 (UIF misrepresentation)
- The Income Tax Act 58 of 1962 (incorrect source declaration / phantom amount)

EXECUTIVE SUMMARY

This disclosure sets out a complete, chronological record from March 2025 to April 2026 of:

1. A systemic clinical governance failure (Gexus-DMS integration gap) affecting GEMS beneficiaries, including a confirmed case involving a 75-year-old member.
2. My protected disclosures (formal, documented, and statutorily mandated) of that failure.
3. Immediate and escalating retaliation — including insubordination allegations, gender identity discrimination ('Wilma'), a recorded admission of retaliatory intent, a sworn false affidavit attacking my psychiatric condition, and institutional silence.
4. An unlawful salary deduction of R14,811.77 (40.47% of monthly salary) while I was on documented psychiatric leave — in breach of BCEA section 34(2).
5. My constructive dismissal on 12 December 2025 — automatically unfair under sections 186(1)(e) and 187(1)(h) of the LRA.
6. Post-employment misconduct by Medscheme / AfroCentric Health, including: · UIF misrepresentation (falsely declaring I resigned rather than constructive dismissal) · Incorrect Certificate of Service (falsely designating me as 'Care Coordinator' not NQF Level 8 Specialist) · Phantom R9,000 amount on my ITA / source declaration that does not appear on any payslip · Failure to remit UIF contributions for June 2025
7. The direct causal link between the above and my current state of destitution, homelessness, loss of SANC registration, and inability to practise nursing.

NOTE: The Vuvuzela Hotline auto-acknowledgement received on 11 April 2026 (reference GEMS-260411-184830-83263) does NOT constitute a substantive response from the GEMS Board. This Final Consolidated Disclosure supersedes and completes all prior communications.

PART ONE: PROFESSIONAL BACKGROUND AND GOVERNANCE ROLE

I am a Registered Nurse Specialist with approximately 16 years of clinical and governance experience.

Qualifications:

- Postgraduate Diploma in Health Services Management (NQF Level 8) — awarded with 90% distinction in Professional Development
- Bachelor of Technology in Nursing (BTech Gen Pn)
- Registered competencies in Psychiatry, Midwifery, and Community Nursing

Employment role at Medscheme (administrator for GEMS):

- Case Manager — GEMS Active Disease Risk Management Unit
- Acting Chairperson, Health and Safety Committee — managing 13 staff across three floors

These roles carried both clinical accountability and formal governance obligations, including the statutory duty to escalate material risks to beneficiaries.

PART TWO: THE GENESIS — INITIAL CONCERNS (MARCH 2025)

On or about 6 March 2025, I identified and raised concerns regarding duplicate LACE files within Medscheme's operational systems. I expressly noted that time spent processing duplicated data entries could instead be used for direct patient engagement with GEMS members requiring clinical intervention.

This was not an administrative complaint. It was a clinical governance concern regarding resource allocation and patient care outcomes.

The response: Intimidation and suppression — not engagement or investigation. This established an early pattern of hostility toward my attempts to address systemic issues.

This constituted either:

- An initial protected disclosure under section 1 of the PDA, or
- At minimum, a precursor disclosure forming part of a continuous course of protected conduct.

PART THREE: MENTAL HEALTH DISCLOSURE AND FAILURE TO ACCOMMODATE (MARCH – MAY 2025)

- 18 March 2025: I was admitted as an inpatient at Akeso Stepping Stones Clinic for severe depression and PTSD (diagnosed by Dr Jacques Malan, Specialist Psychiatrist, FCPSYCH(SA)).
- 16 May 2025: I provided Dr Malan's psychiatric letter to Mapule Mashele (Senior Manager, Medscheme), placing Medscheme on formal written notice of my disability under section 1 of the EEA and my need for reasonable accommodation.

Medscheme failed to provide reasonable accommodation:

- On 16 April 2025, Mary-Ann Jones (Team Leader) stated in writing: 'we do not apply staggering of money owed' — a position later used to justify the unlawful deduction.
- On 20–21 May 2025, I suffered a medical crisis and was required to leave the workplace to obtain medication; Jones instructed me to log unpaid leave — a punitive response to a medical emergency.

This constitutes unfair discrimination on the ground of disability under section 6(1) of the EEA.

PART FOUR: THE UNLAWFUL SALARY DEDUCTION (JULY 2025)

In July 2025, while I was between psychiatric admissions and while Medscheme was in possession of Dr Malan's letter confirming my medical vulnerability, R14,811.77 was deducted from my salary — representing 40.47% of my gross monthly salary.

This deduction was:

- Made without my written consent (contravening BCEA section 34(1))
- In excess of the statutory 25% cap (contravening BCEA section 34(2))
- Made without prior consultation
- In breach of Medscheme's own AOD Policy (which confirms the 25% cap)

On 24 July 2025, Jones shared the AOD Policy confirming the 25% cap. When I pointed out that the deduction was unlawful, Jones replied: 'Hi Wilbur, what is illegal?' — feigning ignorance of the very statutory cap she had just confirmed.

This deduction directly caused:

- My inability to pay SANC annual fees (R870)
- My subsequent deregistration as a nurse
- My inability to pay rent → eviction → homelessness (from 2 April 2026)
- Seizure of personal possessions by loan sharks
- No access to running water since January 2026
- Forced withdrawal of R48,400 in retirement savings to meet basic survival needs

This matter is the subject of a separate BCEA section 77A Labour Court claim (Case No. 2026-077546).

PART FIVE: THE GEXUS-DMS INTEGRATION FAILURE — PRIORITY 1 CLINICAL RISK

5.1 Nature of the Systemic Failure

In October 2025, I identified a critical integration failure between Gexus (claims processing system) and the Disease Management System (DMS). This structural failure caused:

- Automated incorrect non-adherence communications being issued to clinically compliant GEMS members
- Potential misclassification of patient adherence status in clinical records
- Risk of inappropriate clinical decision-making by treating practitioners acting on incorrect data
- Member distress, particularly among elderly and vulnerable populations

A Root Cause Analysis (RCA) was conducted internally. The RCA confirmed the integration gap is structural in nature — not an isolated or incidental error.

5.2 Confirmed Member Impact — 75-Year-Old GEMS Beneficiary

A confirmed case involved a 75-year-old GEMS beneficiary who received an incorrect non-adherence communication despite being fully compliant with prescribed treatment. The member's daughter communicated to Medscheme in writing:

"I request that you get your house in order and update your records. Stop confusing old lady."

This single case demonstrates:

- Failure of data integrity controls prior to member communication
- Failure of verification processes
- Potential reputational, regulatory, and legal exposure for GEMS as the principal scheme
- Possible breach of the Medical Schemes Act's member protection obligations
- Potential breach of POPIA

5.3 My Formal Protected Disclosure (23 October 2025)

On 23 October 2025, I made a protected disclosure to Jones, copied to Lisa Mari (Responsible Pharmacist, Medscheme), identifying the structural integration gap and the risk to GEMS members. This constitutes a protected disclosure under section 1 of the PDA because it related to:

- A failure to comply with a legal obligation (the Responsible Pharmacist's obligation under section 49 of the Pharmacy Act 53 of 1974)
- Conduct likely to endanger the health or safety of any individual (GEMS members receiving incorrect clinical assessments)

I stated in writing: "Given that the integration gap still exists, I will not contact the member at this time."

Jones responded: "The below is noted. I am not in agreement with your feedback though."

Lisa Mari, as Responsible Pharmacist, did not intervene to correct the system error.

5.4 My Priority 1 Material Risk Report to the Board (9 December 2025)

On 9 December 2025, I escalated the matter directly to governance level with a Priority 1 Material Risk Report supported by 38 attachments, including the RCA and all member communication records. This report was submitted because operational management had failed to resolve the integration gap and had instead instructed me to contact the vulnerable member on incorrect clinical grounds.

5.5 Active Suppression by Chief Risk Officer

On 18 December 2025, the Chief Risk Officer (Monwabisi Kula) issued an email that I characterise as the active suppression of my Priority 1 Material Risk Report — constituting primary evidence of wilful governance misconduct. This email is available to investigators.

PART FIVE (A): IMPACT ANALYSIS – CLINICAL, FINANCIAL, AND REGULATORY

A5.1 The "Priority 1" Systemic Failure: 7-Year Acute Benefit Drain

The integration gap between the **Gexus (Claims)** and **DMS (Disease Management)** systems is not a dormant IT glitch; it is an active mechanism of member harm and financial leakage.

- **Systemic Benefit Misallocation:** For at least seven years, the failure to link these systems has caused chronic managed care costs (e.g., pathology and monitoring) to be incorrectly deducted from members' **Acute Benefits**.
- **The "6-Month Switch" Lag:** The system fails to timely realign benefits to the correct chronic pool. This lag means members are "blindly" exhausted of their acute funds before the scheme corrects the allocation.
- **Member Financial Exposure:** Once the acute benefit is prematurely depleted due to this misallocation, members are forced to pay **out-of-pocket co-payments** for genuine acute emergencies.
- **Clinical Data Integrity:** The ADRM unit is operating on clinically false data, leading to "phantom" non-adherence alerts. This results in the harassment of compliant, vulnerable members (e.g., Case GEMS 000655789) while genuine high-risk patients remain unmonitored due to the system "blind spot."

A5.2 Regulatory Liability and Pharmacy Act Contraventions

This integration failure creates direct legal and ethical liability for clinical and pharmaceutical leadership under South African law.

- **Responsible Pharmacist (RP) Liability (SAPC):** Under the **Pharmacy Act 53 of 1974** and SAPC *Good Pharmacy Practice* (GPP) rules, the RP is personally responsible for the accuracy of information. Knowingly maintaining a

system with a 6-month data lag forces dispensing pharmacists to provide **inaccurate communications** to members, which is a breach of the SAPC Code of Conduct.

- **Statutory Fraud & "Benefit Shifting"**: Correcting a system error by allowing chronic claims to drain acute pools constitutes a misrepresentation of services to the scheme and the member. This places pharmacy licenses at risk of revocation for "claiming for services not rendered" or "misclassification of claims."
- **SANC Professional Ethics (Nursing Act 33 of 2005)**: The directive from the Head Strategic Team to utilize a "3-month methodology" to ignore the gap—and the subsequent instruction to contact a member based on known false data—is a mandate for **professional negligence**. It forced clinical staff to violate the "Do No Harm" principle and the duty to maintain accurate clinical records.

PART SIX: RETALIATION FOLLOWING PROTECTED DISCLOSURES

Following my ethical refusal and protected disclosures, I experienced the following retaliatory conduct:

6.1 "Wilma" — Gender Identity Discrimination (23 September 2025)

Immediately after I proposed a clinical filtering strategy, Jones posted a message in a team-wide Microsoft Teams channel in which she:

- Referred to me as "Wilma" — a feminised version of my name, knowing I am gay
- In the same message, prohibited my filtering strategy with "NO FILTERING" highlighted in red

This constitutes harassment on grounds of gender identity and sexual orientation under section 6(3) of the EEA.

My formal complaint to HR (25 September 2025) was whitewashed — the investigation was closed on 1 October 2025 with the finding that "Wilma" was a "typo." This finding is manifestly inadequate.

6.2 Recorded Admission of Retaliatory Intent (31 October 2025)

In a meeting that I lawfully recorded, Jones stated:

"You made a case of harassment against me, so I can make a case of harassment against you. You misspelled my name in an email — go look."

Jones further warned me to "think of the ripple effect" of my actions as a person who had made formal complaints and protected disclosures.

This constitutes direct contemporaneous evidence of a causal nexus between my protected activities and retaliatory intent.

6.3 Insubordination Allegation

I was formally accused of insubordination for declining to contact the 75-year-old member on clinically incorrect grounds.

6.4 Institutional Silence (November – December 2025)

- 2 November 2025: Formal grievance filed against Jones — over 30 days of institutional silence (no acknowledgment, no investigation, no response)
- 4 December 2025: Escalation to executive leadership (Gerald Van Wyk, CEO of AfroCentric Health) with 32-page evidence bundle — de-escalated back to the department I was complaining about

- 9 December 2025: Chief Risk Officer acknowledged receipt but stated interim feedback would only be provided in January 2026 due to the "festive break" — a deliberate delay tactic given my humanitarian urgency

6.5 Sworn False Affidavit Attacking My Psychiatric Condition (10 December 2025)

On 10 December 2025, Medscheme / AfroCentric Health, through their legal representative Xolile Vincent Manyathi, filed an opposing affidavit in the CCMA that:

- Dismissed my diagnosed depression and PTSD as a "cynical excuse" and an "afterword"
- Labelled me as having an "untruthful character"
- Described my clinical conduct as "sloppiness" and a "fishing expedition"

This affidavit was filed under oath before a Commissioner of Oaths. I have never met, engaged with, or been observed by Manyathi.

This constitutes:

- Discrimination on grounds of disability under section 6(1) of the EEA
- A violation of my right to dignity under section 10 of the Constitution
- Evidence of bad faith that eliminated any objective possibility of restoring the employment relationship

PART SEVEN: CONSTRUCTIVE DISMISSAL (12 DECEMBER 2025)

On 11 December 2025, I gave formal notice of resignation, effective 12 December 2025, citing the employer's sworn affidavit as the final intolerable act.

The employment situation had become objectively intolerable due to the cumulative effect of:

- The unlawful 40.47% salary deduction while on psychiatric leave
- Discrimination and harassment ("Wilma") followed by a whitewash investigation
- Retaliation and insubordination labelling following a bona fide patient safety disclosure
- The recorded admission of retaliatory intent
- Over 30 days of institutional silence on formal grievances
- The executive "festive break" delay and Chief Risk Officer gatekeeping
- The sworn, defamatory affidavit attacking my psychiatric condition
- The Board's failure to respond substantively to the Priority 1 Material Risk Report

My resignation constitutes a constructive dismissal under section 186(1)(e) of the LRA.

The dominant reason for the dismissal is retaliation for protected disclosures — making it automatically unfair under section 187(1)(h) of the LRA. Alternatively, it is automatically unfair under section 187(1)(f) (discrimination on grounds of disability).

This matter is currently before the Labour Court, Cape Town, as part of my Statement of Claim filed 9 April 2026.

PART EIGHT: POST-EMPLOYMENT MISCONDUCT AND MISREPRESENTATION

8.1 Statutory Misrepresentation on UI-19 Declaration:

On 19 January 2026, Medscheme Holdings submitted a formal UI-19 declaration to the Department of Employment and Labour.

In this document, the employer certified under "Reason for Termination" that I had "Resigned" (Code 6).

This is a factual misrepresentation of the termination status.

The employer was in possession of my formal notice dated 11 December 2025, which explicitly defined the termination as an Automatically Unfair Constructive Dismissal under section 186(1)(e) and 187(1)(h) of the LRA.

By using "Code 6" (Resignation) instead of "Code 7" (Constructive Dismissal), the employer made a false declaration on a statutory document to the Unemployment Insurance Fund.

This act constitutes a continuation of occupational detriment post-employment, as it was designed to mischaracterize the nature of my departure and potentially obstruct my access to statutory benefits.

8.2 Incorrect Certificate of Service

The Certificate of Service issued to me designates me as "Care Coordinator" — not Care Manager, not Health Services Management Nurse Specialist (NQF Level 8). I was recruited for a Specialist: Care Manager role (three interview invitations, assessment instructions, fingerprint verification all confirm this). My contract was downgraded to "Care Coordinator" at the last minute — a deliberate act of title suppression.

Senior HR Business Partner Cheryl-Dawn Modern confirmed in writing on 19 January 2026 that "naming convention changes [were] still in progress" — an admission that my designation had not been updated to reflect my true role. This contravenes section 42 of the BCEA (duty to provide an accurate certificate of service).

8.3 Phantom R9,000 on ITA / Source Declaration

On my ITA / source declaration (SARS), there appears a phantom amount of approximately R9,000 that does not appear on any of my payslips for the relevant period. I have been unable to reconcile this amount. It appears to be a false declaration to SARS.

Potential explanations include:

- Incorrect reporting by the employer
- Deliberate misrepresentation affecting my tax liability
- An amount deducted but not reflected on payslips

I request that the GEMS Board, as principal, require Medscheme to provide a sworn explanation of this discrepancy and to correct any incorrect SARS declaration.

8.4 UIF Non-Remittance for June 2025

UIF contributions were deducted from my June 2025 salary, but Medscheme failed to declare my employment for that month. The uFiling portal continues to reflect "No Information" for June 2025.

This constitutes:

- A contravention of section 5 of the Unemployment Insurance Contributions Act 4 of 2002

- Potential fraud under section 68 of the Unemployment Insurance Act 63 of 2001

A criminal case has been registered at SAPS Delft (CAS 705/3/2026) and transferred to SAPS Woodstock on 25 March 2026 for investigation.

PART NINE: THE DIRECT CAUSAL LINK BETWEEN RESPONDENTS' CONDUCT AND MY CURRENT STATE

The cumulative consequences of the unlawful deduction, retaliation, and institutional failures are severe and ongoing:

Consequence	Direct Cause
Loss of SANC professional registration (unable to pay R870)	Unlawful R14,811.77 deduction
Homelessness (from 2 April 2026)	Financial destruction from unlawful deduction
No access to running water (since January 2026)	Financial destruction
Seizure of personal possessions by loan sharks	Financial destruction
Forced withdrawal of R48,400 in retirement savings	Financial destruction
Severe depression and PTSD (Dr Jacques Malan)	Directly causally linked to occupational circumstances
Loss of CPUT Postgraduate Diploma in Occupational Health Nursing (2026 acceptance lost)	Inability to pay registration fees

I am a Registered Nurse Specialist who cannot practise nursing because I could not pay R870 — directly caused by an unlawful deduction of R14,811.77.

PART TEN: EXHAUSTION OF INSTITUTIONAL RECOURSE — NO SUBSTANTIVE REMEDY

I have approached the following institutions. None provided substantive relief:

Institution	Outcome
CCMA	Certificates issued; commissioner bias documented
Labour Court	Portal obstruction; registrar non-response documented
Western Cape High Court (Case No. 2026-066952)	Struck from roll; Rule 42 rescission pending
Council for Medical Schemes	Jurisdiction declined twice
B-BBEE Commission (Case No. 5/02/2026)	Investigation underway
SAPS Criminal case CAS 705/3/2026	Registered
Department of Employment & Labour	Seven weeks of silence
SAHRC	Seriousness acknowledged; redirected

JSE Issuer Regulation	Jurisdiction declined
SANC	Deregistration confirmed (occupational detriment under PDA)

The Vuvuzela Hotline auto-acknowledgement (11 April 2026) does not change this. It is not a substantive response from the GEMS Board.

PART TEN: CORPORATE GOVERNANCE BREACHES & PUBLIC MISREPRESENTATION

10.1 Material Omissions in the 8 April 2026 Integrated Report

On 31 March 2026, the AfroCentric Board approved the Integrated Annual Report, which was subsequently published to the JSE and the public on 8 April 2026. This report serves as a “sworn statement” to the market. However, a forensic comparison between the report’s claims and the physical record reveals a pattern of administrative misrepresentation: Clinical Liability (The Gexus-DMS “Benefit Drain”): * The Claim: The report emphasizes “Clinical Innovation” and “Value-Based Care.”

The Omission: It fails to disclose the 7-year technical integration gap (2017–2024). This gap caused chronic pathology and medicine claims to be processed against Acute/Day-to-Day benefits, prematurely depleting member savings for costs that should have been covered by the Chronic pool. This is a material clinical risk affecting 1.9 million lives.

Securities Misrepresentation (Non-Disclosure of Material Litigation):

The Claim: The report asserts effective governance and a stable leadership environment.

The Omission: The report omits the Section 162 Director Delinquency Application (Filing Ref: HCGS951842) and the B-BBEE Commission Fronting Investigation (Case 5/02/2026).

The Breach: Under JSE Listings Requirements (Section 7.D.11), “material legal proceedings” must be disclosed. By publishing a “clean” report after being formally notified of these filings via the JSE Sponsor (Questco) in March 2026, the Board has provided a false sense of security to investors and the GEMS Board.

Ethical Collapse & Suppression of Risk:

The Claim: The report boasts of a “Zero Tolerance for Unethical Conduct” and robust “Whistleblower Protection.”

The Reality: The Board suppressed the CRO1 directive (18 December 2025) and the Root Cause Analysis (RCA). The directive from the Chief Risk Officer to “stop emailing directors” regarding a Priority 1 (P1) risk—and the instruction to a Specialist Nurse to probe a member for “non-adherence” while knowing the system was at fault—proves an institutional culture of non-compliance.

10.2 Financial Perjury: SARS & UIF Discrepancies

The integrity of the Group’s financial reporting is compromised by documented statutory reporting errors:

The “Phantom” Overtime: The company reported R8,863.15 in taxable overtime to SARS for the 2024/2025 period.

The Contradiction: This was reported despite a written directive from Mary-Ann Jones prohibiting the performance of overtime. Reporting false payroll data to SARS and the Department of Employment and Labour (as evidenced by the incorrect Code 6 on the UI-19 form signed 19 January 2026) constitutes a criminal offense under the Tax Administration Act.

10.3 The Strategic “Event Horizon” for the GEMS Board

The 8 April report was published after the Board was served with the 17 March 2026 Procedural Notice. They cannot claim ignorance. By signing their names to these omissions, the directors have moved from “oversight failure” to “Statutory Default.”

Warning to GEMS: GEMS is required to report its findings to the Council for Medical Schemes (CMS) by 30 April 2026. If GEMS relies on the AfroCentric Integrated Report of 8 April, it will be adopting a factually compromised narrative. This places the GEMS Board in a position of secondary misrepresentation to the CMS, potentially triggering regulatory intervention against the Scheme itself.

PART ELEVEN: SPECIFIC REQUESTED ACTIONS FROM THE GEMS BOARD

I respectfully request that the GEMS Board of Trustees:

A. Clinical Governance

8. Commission an independent investigation into the Gexus-DMS integration failure.
9. Obtain and review the Root Cause Analysis conducted by Medscheme and verify whether a remediation plan was implemented.
10. Assess the full extent of member impact, including identification of all potentially affected beneficiaries.
11. Verify whether this matter has previously been disclosed to the Board — and if not, why not.

B. Administrator Oversight

12. Review Medscheme's risk escalation and reporting protocols to GEMS under the Service Level Agreement.
13. Review the conduct surrounding the CRO email of 18 December 2025 and determine whether risk suppression occurred.
14. Require Medscheme to confirm in writing whether any disciplinary or remedial action has been taken against any employee responsible for the suppression.

C. Post-Employment Misconduct

15. Require Medscheme to correct my Certificate of Service to reflect my true role (Health Services Management Nurse Specialist / Case Manager, NQF Level 8).
16. Require Medscheme to correct the UIF declaration — from false "resignation" to constructive dismissal — and to remit all outstanding UIF contributions.
17. Require Medscheme to provide a sworn explanation of the phantom R9,000 on my ITA / source declaration and to correct any incorrect SARS filing.

D. Accountability and Remedy

18. Provide me with a substantive written response within 14 days of this Final Consolidated Disclosure.
19. Confirm in writing whether any of the matters disclosed herein will be referred to the CMS, HPCSA, or SANC for further investigation.
20. Take steps to remedy the occupational detriment I have suffered, including engaging with SANC to explore reinstatement of my registration given the direct causal link to the unlawful deduction.

PART TWELVE: LEGAL PROTECTION AND NOTICE OF ESCALATION

This disclosure is made in good faith, under the full protections of the Protected Disclosures Act 26 of 2000.

I am aware that:

- Section 3 of the PDA prohibits any occupational detriment against me for making this disclosure.
- Section 4 of the PDA provides for civil remedies, including compensation for any loss suffered.
- Section 187(1)(h) of the LRA makes dismissal for making a protected disclosure automatically unfair.

If the GEMS Board fails to provide a substantive response within 14 days of this disclosure, I will escalate this matter without further notice to:

- The Parliamentary Portfolio Committee on Health
- The Public Protector of South Africa
- The Council for Medical Schemes (re-submission with full record)
- The South African Human Rights Commission (re-submission with full record)

-
- The Minister of Health
 - Appropriate public interest and media platforms

PART THIRTEEN: DECLARATION

I affirm that the contents of this Final Consolidated Protected Disclosure are true and correct to the best of my knowledge, information, and belief. I am aware of the legal consequences of making a false statement.

This disclosure is accompanied by the following supporting documents (available on request):

- Labour Court Statement of Claim (9 April 2026) — 19 pages, 148-entry Chronological Register
- Supporting Testimony (2 April 2026) — personal impact and retaliation
- All payslips, correspondence, RCA, and CRO email
- B-BBEE Commission case confirmation (5/02/2026)
- SAPS criminal case confirmation (CAS 705/3/2026)
- SAHRC acknowledgment
- CMS referral correspondence
- SANC deregistration confirmation

Yours faithfully,

SIGNATURE: _____

Wilbur William Matthee

Registered Nurse Specialist | Forensic Governance Practitioner

Founder, EthicHawks Forensic Governance Consultancy

Former Case Manager — GEMS Active Disease Risk Management Unit

Former Acting Chairperson, Health and Safety Committee

Email: info@ethichawks.co.za

Date: 11 April 2026

Place: Cape Town